



Laboratory Investigation Report

Patient Name	: Miki Tiwari	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 29 Y 9 M 18 D /F	OP/IP No	: OP/98702
Max ID/Mobile	: 299885/8752005200	Collection Date/Time	: 25/Aug/2021 04:24PM
Lab ID	: 1266082100592	Receiving Date	: 25/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 25/Aug/2021

Clinical Biochemistry

KFT Profile with Calcium,Uric Acid*, Serum

Date	25/Aug/2021 04:24PM	Unit	Bio Ref Interval
Urea Urease, UV	17.0	mg/dL	17.0 - 43.0
Creatinine Rate-Jaffe	0.76	mg/dl	0.61 - 1.24
eGFR MDRD	89.48	ml/min/1.73 m ²	
Uric Acid Uricase, Colorimetric	6.49	mg/dL	2.6 - 6.0
Calcium (Total) Arsenazo III	9.29	mg/dL	8.8 - 10.6
Sodium ISE Indirect	135.6	mmol/L	136 - 146
Potassium ISE indirect	4.62	mmol/L	3.5 - 5.1
Chloride ISE indirect	103.0	mmol/L	101 - 109
Bicarbonate Enzymatic	23.6	mmol/L	21 - 31

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs ≤ 60 ml / min / 1.73 m². MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

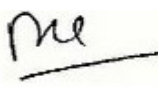
<15 ml / min / 1.73 m² - Kidney failure

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
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CIN No.: L72200MH2001PLC322854

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Serology

Test Name	Result	Unit	Bio Ref Interval
Rapid Card Test - HCV*, Serum Immunochromatography			
HCV Card Test	Negative		

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR
A non reactive result does not exclude the possibility of exposure to or infection with HCV.
Patients with auto immune liver disease may show falsely reactive results

Kindly correlate with clinical findings

*** End Of Report ***



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Clinical Biochemistry

Blood Sugar Fasting, Fluoride Plasma

Date	25/Aug/2021 04:22PM	Unit	Bio Ref Interval
Glucose (Fasting) Hexokinase	92	mg/dL	74 - 99





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Clinical Biochemistry

Liver Function Test Profile, Serum

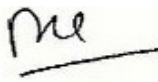
Date	25/Aug/2021 04:24PM	Unit	Bio Ref Interval
Total Protein	7.21	g/dL	6.6 - 8.3
Biuret			
Albumin	4.1	g/dL	3.5 - 5.2
Bromocresol Green (BCG)			
Globulin	3.1	g/dl	2.3 - 3.5
Calculated			
A.G. ratio	1.3		1.2 - 1.5
Calculated			
Bilirubin (Total)	0.25	mg/dL	0.3 - 1.2
DPD			
Bilirubin (Direct)	0.05	mg/dL	0.0 - 0.2
Diazotization			
Bilirubin (Indirect)	0.20	mg/dL	0.1 - 1.0
Calculated			
SGOT- Aspartate Transaminase (AST)	40	U/L	< 50
UV without P5P			
SGPT- Alanine Transaminase (ALT)	53	IU/L	< 35
UV without P5P			
Alkaline Phosphatase	87	IU/L	30 - 120
PNPP, AMP Buffer			
GGTP (Gamma GT), Serum	42.0	IU/L	< 38
Enzymatic Rate			

Kindly correlate with clinical findings

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Clinical Pathology

Urine Routine And Microscopy

Date	25/Aug/2021	21/Apr/21	07/Nov/20	Unit	Bio Ref Interval
	04:27PM	06:09PM	05:03PM		

Macroscopy

Reflectance photometry

Colour	Yellow	Yellow	Pale Yellow		Pale Yellow
PH	5.0	5.0	5.5	..	5-6
Specific Gravity	1.020	1.018	1.020		1.015 - 1.025
Protein	Trace	Trace	Nil		Nil
Glucose	Nil	Nil	Nil		Nil
Ketones	Nil	Nil	Nil		Nil
Blood	Nil	Trace	+		Nil
Bilirubin	Nil	Nil	Nil		Nil
Urobilinogen	Normal	Normal	Normal		Normal
Nitrite	Negative	Negative	Negative		

Microscopy

Light Microscopy/Image capture microscopy

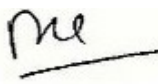
Red Blood Cells (RBC)	Nil	1 - 2	10-12	/HPF	Nil
White Blood Cells	3 - 5	1 - 2	4-5	/HPF	0.0-5.0
Squamous Epithelial Cells	3 - 5	2 - 3	2-3	/HPF	
Cast	Nil	Nil	Nil	/LPF	Nil
Crystals	Nil	Nil	Nil	..	Nil
Bacteria	Nil	Nil	Nil	/HPF	Nil

Kindly correlate with clinical findings

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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma, Sodium Citrate

Date	25/Aug/2021 04:24PM	Unit	Bio Ref Interval
APTT	25.5	Sec	24.0 - 36.0
Control	29.2	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma

Date	25/Aug/2021 04:20PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	11.4	Sec	9.8 - 13.3
MNPT Value	11.5	Sec	
INR	0.99		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.

CBC (Complete Blood Count), Whole Blood EDTA

Date	25/Aug/2021	21/Apr/21	07/Nov/20	Unit	Bio Ref Interval
	04:22PM	06:09PM	05:03PM		



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Hematology

Haemoglobin	11.7	11.1	11.67	g/dl	12.0 - 15.0
Packed Cell, Volume	37.1	35.6	35.0	%	36-46
Calculated					
Total Leucocyte Count (TLC)	4.3	7.1	5.49	10~9/L	4.0-10.0
Electrical Impedance					
RBC Count	4.36	4.43	4.37	10~12/L	3.8-4.8
Electrical Impedance					
MCV	85.1	80.4	80.0	fL	83-101
Electrical Impedance					
MCH	26.8	25.1	26.7	pg	27-32
Calculated					
MCHC	31.5	31.2	33.3	g/dl	31.5-34.5
Calculated					
Platelet Count	175	201	240.3	10~9/L	150-410
Electrical Impedance					
MPV	12.1	11.5	11.85	fl	7.8-11.2
Calculated					
RDW	17.1	15.3	15.0	%	11.5-14.5
Calculated					

Differential Cell Count

VCS / Light Microscopy

Neutrophils	38.3	51.6	55	%	40-80
Lymphocytes	47.8	36.5	38	%	20-40
Monocytes	9.9	9.6	06	%	2-10
Eosinophils	3.2	1.0	01	%	1-6
Basophils	0.8	1.3	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	1.65	3.66	3.02	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.1	2.6	2.1	10~9/L	1.0-3.0
Absolute Monocyte Count	0.43	0.68	0.33	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.14	0.07	0.05	10~9/L	0.02-0.5
Absolute Basophil Count	0.03	0.09		10~9/L	0.02-0.1



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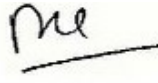
Hematology

Kindly correlate with clinical findings

*** End Of Report ***



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Immunoassay

TSH,Serum

Date	25/Aug/2021 04:27PM	20/Jul/21 10:31AM	21/Apr/21 06:09PM	22/Dec/20 10:53AM	07/Nov/20 05:03PM	Unit	Bio Ref Interval
Thyroid Stimulating Hormone CLIA	0.32	2.69	5.94	6.6	2.6	µIU/mL	0.34 - 5.6

Interpretation

Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

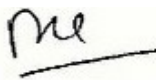
Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

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Serology

Test Name	Result	Unit	Bio Ref Interval
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Hepatitis B Surface Antigen,Serum

CLIA

HBsAg Test Value	0.18
------------------	------

Ref. Range

Negative	< 0.90
Borderline	0.90 - 5.0
Positive	> 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.
- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.



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Serology

Test Name	Result	Unit	Bio Ref Interval
VDRL/RPR, Serum			
RPR(Syphilis) Slide Flocculation	Non Reactive		

Comment**Interpretation**

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

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